

August 20, 2026

Aetna
Provider Resolution Team
P.O. Box 14020
Lexington, KY 40512

RE: Formal Appeal of Claim Denial — Marc Corkery (MRN MUSC8042994), Claim MUSC-76E3-1

Patient	Marc Corkery (DOB 1977-08-10)
MRN	MUSC8042994
Member ID / Group	AET942376912 / GRP-91373
Claim number	MUSC-76E3-1
Date of service	2026-05-22
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	R69 — Illness, unspecified
Denied amount	\$516.06
Denial code	CARC 204 / RARC N130 — This service/equipment/drug is not covered under the patient's current benefit plan.
Appeal deadline	2026-12-09

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this Level 1 reconsideration on behalf of Marc Corkery (Member ID: AET942376912, Group: GRP-91373, Date of Birth: August 10, 1977) regarding the denial of Claim MUSC-76E3-1 for services rendered on May 22, 2026, at MUSC Health East Cooper Medical Center in Mount Pleasant, South Carolina. The claim was submitted on June 2, 2026, for CPT code 99214 (office/outpatient visit, established patient, moderate complexity), billed at \$516.06. Aetna denied the claim in full on June 12, 2026, citing CARC 204 ("This service/equipment/drug is not covered under the patient's current benefit plan") and RARC N130 ("Consult plan benefit documents/guidelines for information about restrictions for this service"), with the payer remark stating: "Service is excluded under the member's plan benefit document." MUSC Health respectfully disputes this determination and requests immediate reconsideration and payment of the allowed amount of \$314.27.

CLINICAL BACKGROUND

Mr. Corkery is a 49-year-old male established patient with an active diagnosis of obesity (ICD-10 E66.9), documented as ongoing since 2015, with a recorded body mass index exceeding 30 kg/m² and body weight of approximately 103.5 kg. He also carries active inhaler-dependent respiratory management, as evidenced by long-standing prescriptions for fluticasone propionate and albuterol metered dose inhalers. The May 22, 2026, encounter was billed under primary diagnosis R69 with secondary diagnosis E66.9 and was performed by rendering provider Sarah E. Whitmore, MD (NPI 1447382910) in an on-campus outpatient hospital setting (Place of Service 22). The visit was coded at the 99214 level, consistent with an established patient encounter of moderate medical decision-making complexity, which is appropriate given the patient's active, chronic conditions requiring ongoing clinical management.

BASIS FOR APPEAL

The denial characterizing this evaluation and management service as a non-covered benefit is not supported by the facts of this claim. CPT code 99214 represents a standard outpatient evaluation and management service for an established patient — one of the most fundamental categories of covered medical care under virtually all commercial health benefit plans. Mr. Corkery's Aetna Advantage plan carries a coverage period running from January 1, 2024, through December 31, 2026, confirming that active, valid coverage was in place on the date of service. The claim does not involve a service category that is commonly subject to blanket exclusions, such as cosmetic procedures, experimental treatments, or non-physician services. Rather, it reflects a face-to-face physician visit for the ongoing management of a documented, active chronic condition.

Aetna's remittance does not identify any specific exclusionary provision within the plan benefit document that would apply to a routine outpatient physician visit of this nature. A categorical denial under CARC 204 without citation of a specific applicable exclusion is insufficient to sustain a coverage denial for a medically necessary evaluation and management service. Under generally accepted standards governing commercial health plan administration, payers are obligated to provide a clear and specific basis for any benefit exclusion determination. The denial as issued does not meet that standard.

Furthermore, the allowed amount of \$314.27 was calculated on the claim, which indicates that Aetna's own adjudication system recognized the service as a payable code at a contracted rate — an action inconsistent with a determination that the service is categorically non-covered.

REQUESTED ACTION

MUSC Health respectfully requests that Aetna reverse the denial of Claim MUSC-76E3-1, identify the specific plan exclusion language relied upon if the denial is to be upheld, and issue payment at the previously calculated allowed amount of \$314.27 for CPT code 99214 rendered on May 22, 2026. Should Aetna require additional clinical documentation to support the medical necessity of this encounter, MUSC Health will provide the complete visit record upon request. We ask that this reconsideration be completed within the timeframe required under applicable prompt-pay and appeals-rights obligations.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record